

MRI Pelvis with Contrast -Radiology Report

GI Associates of Boston | Dr. Elena Hartley, MD

Patient: Jane Doe | DOB: 1988-03-14 | MRN: 00142837

Exam date: 2026-04-20 Radiologist: Dr. K. Singh, MD Accession: MR-2026-03841

Clinical indication: Active ulcerative colitis, rule out perianal Crohn's, abscess

TECHNIQUE

MRI pelvis with and without IV gadolinium contrast. Multiplanar T1, T2, and DWI sequences.

FINDINGS

Colon: Mural thickening of the rectum and sigmoid colon up to 8mm. Increased T2 signal within the bowel wall consistent with edema and active inflammation. Mucosal enhancement pattern on post-contrast imaging confirms active colitis. No skip lesions. No transmural involvement.

Perirectal region: No fistula tracts, abscesses, or sinus tracts identified. Mesorectal fat stranding minimal. No lymphadenopathy exceeding 10mm.

Small bowel: Unremarkable. No terminal ileal thickening.

Other structures: Uterus and ovaries unremarkable. Bladder normal. No free fluid.

IMPRESSION

- Active proctosigmoiditis consistent with left-sided ulcerative colitis (E2).
- No perianal fistula, abscess, or features of Crohn's disease.
- No evidence of toxic megacolon or perforation.